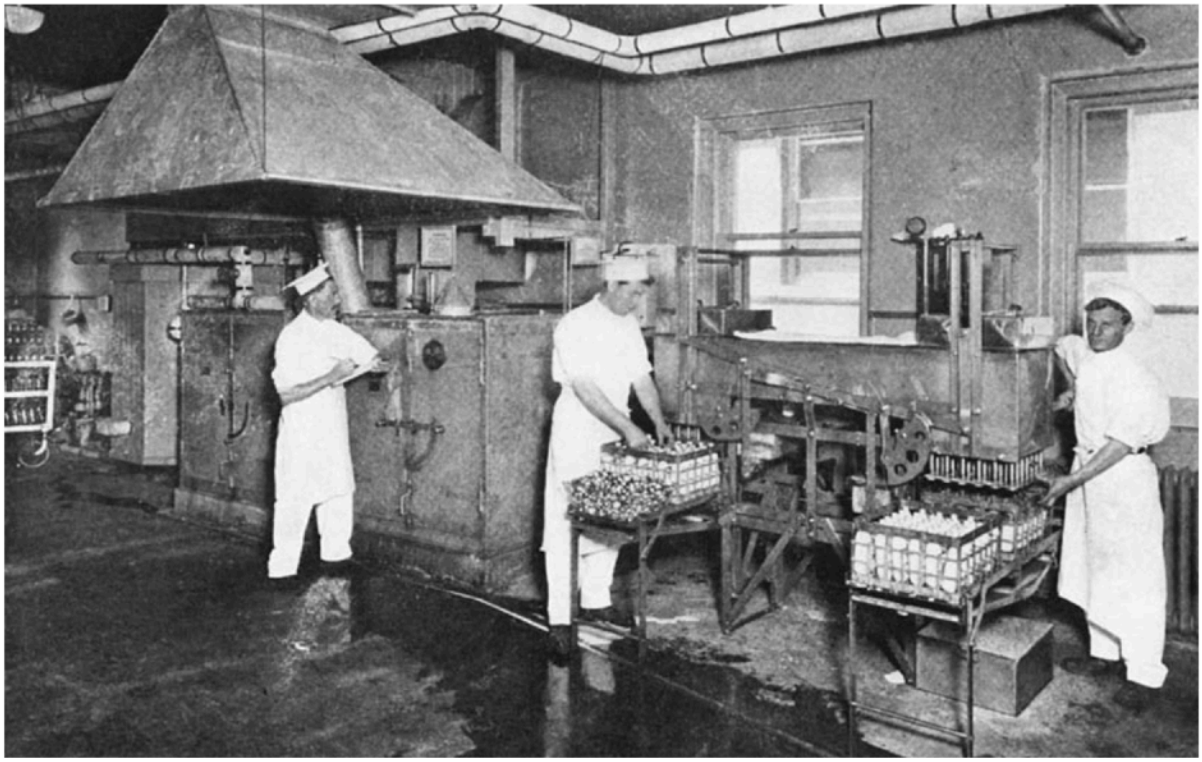

FIGURE 2.22. A pasteurizing room was a distinct feature of children's hospitals. Hospital for Sick Children, Toronto.

4. In Chapter 3, **Nurses**, I felt enraged by the gendered inequalities visible in the architecture. The pain of (nurses) being constantly threatened with expulsion made this building, seemingly created to attract the best talent, sound horrific. It also felt like a punch in the gut that on the one hand here they are attracting all of these "best-of-the-best" nurses with independence and for them to watch the housemen effortlessly move through the building with the true freedom that was sold to them. I hope it is poetic of the progress we have made that the section of the hospital that housed the student nurses is indistinguishable from the rest of institution. That it means that we have improved our struggle for visibility and autonomy and not that we are blindly reliving history. Based on my own life experiences, coupled with reading your more recent article, I feel as though it is where curiosity and imagination are absent that we are reliving these nurses' gendered history.

5. In Chapter 4, **Architects and Doctors**, the life of Stevens' sounded like the most incredible career description: [1] to get to be interdisciplinary and combine architecture with health care, [2] his positive 21-year working relationship with Frederick Lee, [3] traveling to so many cities in European to affirm his hospital knowledge, [4] the sheer volume of 100 hospitals they designed, [5] getting to incorporate his love of hydrotherapy. Also, Minnie Goodnow stood out as a historical heroine to me, I read the quote "A knowledge of plumbing and painting was not harder to acquire than a knowledge of silks or clothes," with an early twentieth century poise, grace and confidence. Lastly, it was fascinating to find out that learning from observing stems from their era.

6. In Chapter 5, **Modernism**, it was magical reading about the many ways that the historic and modern were integrated. The reference to children's hospitals at the end was delightful. I marveled at Steven's sophisticated utilization of psychology to enhance his design plans and the tenderly humorous reference to his later "unusual ambivalence" towards the adoption of his dogmatic 80% design plan in the majority of newer large hospitals. The verbal dissection of the last section, titled "Coda", the root of the word being the Latin 'cauda' or tail, etymologically left off on a lively lyricism, a moving into the future from what exists, as related to the extension of the tail, or to continuing the literary thread.